

SYNTHETIC CLINICAL PROTOCOL

**A Randomized, Double-Blind Study of Synthetic Investigational
Compound AUR-101 in Healthy Adult Participants**

Study ID: AURORA-101

Protocol Version: 2.0

Document ID: AURORA-101-PROT-V2.0

Sponsor: Synthetic Aurora Research Consortium (fictional)

All names, contacts, sites, participants, and data in this document are fictional. This document is a proof-of-concept fixture for Protocol 215 and must never be used for real clinical, regulatory, or patient-care purposes.

1. Synopsis [SEC-SYNOPSIS]

This synthetic protocol describes a randomized, double-blind evaluation of investigational compound AUR-101 in healthy adult participants. The study is entirely fictional and exists only to exercise Protocol 215 amendment-preflight systems.

Participants receive a single oral dose of AUR-101 or matching placebo on Day 1 under clinic supervision, with outpatient follow-up through Day 8. Safety, tolerability, and pharmacokinetic (PK) characterization are the primary scientific aims of this fixture narrative.

Operational contacts, laboratory vendors, and electronic data-capture (EDC) fields referenced herein use reserved example.test domains and synthetic identifiers only.

2. Objectives [SEC-OBJECTIVES]

Primary objective: characterize the single-dose safety and tolerability profile of synthetic AUR-101 in healthy adults under controlled clinic conditions.

Secondary objective: describe plasma concentration–time characteristics of AUR-101 using the Day 1 PK sampling schedule defined in Section 7.1.

Exploratory objective: evaluate operational readiness of site workflows for sample handling, ECG capture, and EDC data entry in a synthetic environment.

3. Study Design [SEC-DESIGN]

Design: randomized, double-blind, placebo-controlled, single-dose study. Randomization and treatment assignment are fictional constructs and must not be interpreted as medical guidance.

Setting: three fictional investigative sites (Phoenix SITE-001, Boston SITE-002, Seattle SITE-003) operating under Protocol Version 1.0 unless an approved amendment activates a later version.

Duration: screening up to 14 days, Day 1 dosing visit, and Day 8 follow-up. Historical completed visits remain immutable once recorded in the synthetic Trial Twin.

4. Eligibility Summary [SEC-ELIGIBILITY]

Key inclusion (synthetic): adults 18–55 years; able to comply with clinic visit schedule; willing to provide informed consent using the version active at the site.

Key exclusion (synthetic): known hypersensitivity to study materials; clinically significant ECG abnormality at screening per investigator judgment in this fictional narrative; inability to complete fasting requirements.

Eligibility determinations in this fixture are narrative only and must never be used to enroll real participants.

5. Treatment Overview [SEC-TREATMENT]

Investigational product: synthetic AUR-101 oral capsule or matching placebo administered once on Day 1 under supervision.

Dose time is scheduled per participant calendar in the synthetic study state. Sites must not alter completed dosing records.

Concomitant medications and rescue procedures are out of scope for this fixture beyond documenting operational impacts of amendment changes.

6. Schedule of Activities [SEC-SOA]

The Schedule of Activities summarizes required assessments by visit. Cells marked X are required. PK rows mirror Section 7.1.

Activity	Screening	Day 1	Day 8	Notes
Informed consent	X			Version active at site
Eligibility review	X			Synthetic criteria only
Physical exam	X	X	X	
Vital signs	X	X	X	
Baseline ECG	X	X		See Section 8
Conditional repeat ECG		X		If AURORA ECG review trigger met
Dosing (AUR-101 or placebo)		X		Clinic supervised
PK sample pre-dose		X		Section 7.1
PK sample 0.5 h		X		Section 7.1
PK sample 1 h		X		Section 7.1
PK sample 2 h		X		Section 7.1
PK sample 4 h		X		Section 7.1
PK sample 6 h		X		Section 7.1 — amendment
Post-dose fasting window		X		Through 4 hours
Laboratory samples	X	X	X	Central lab
Adverse event review		X	X	

7. Day 1 Visit [SEC-DAY1]

Day 1 is an inpatient clinic visit. Participants arrive fasting as instructed, complete pre-dose procedures, receive study treatment, and remain for the PK window defined in Section 7.1.

Sites must confirm courier departure times and validated overnight storage capability before releasing time-sensitive PK samples.

7.1 Pharmacokinetic Sampling [SEC-PK]

Collect plasma PK samples relative to dosing on Day 1 at the following timepoints:

- Pre-dose
- 0.5 hour post-dose
- 1 hour post-dose
- 2 hours post-dose
- 4 hours post-dose
- 6 hours post-dose

7.2 Post-Dose Fasting [SEC-FASTING]

Participants must remain fasting after dosing through 4 hours post-dose, except for water as permitted by site staff.

Meal service may begin only after the 4-hour fasting window is complete.

8. Electrocardiogram Schedule [SEC-ECG]

Obtain a baseline 12-lead ECG at screening and a pre-dose ECG on Day 1.

Conditional repeat ECG requirement: if the fictional protocol-defined AURORA ECG review trigger is met on Day 1, obtain one additional 12-lead ECG before clinic discharge.

The AURORA ECG review trigger is a synthetic rule for this fixture (for example, a machine-read QTc interval above the protocol narrative threshold). Autonomous clinical activation of treatment decisions remains prohibited.

9. Laboratory Processing [SEC-LAB]

Process laboratory and PK samples according to the central laboratory manual. Maintain the sample-processing window of 60 minutes from collection to freezer placement unless otherwise specified.

Record sample identifiers and collection times in the EDC. Do not invent storage capabilities that the site Twin does not possess.

9.1 Central Laboratory Contact [SEC-LAB-CONTACT]

Central laboratory contact email: lab-v2@example.test

Central laboratory phone (fictional): +1-555-010-1001

Ship samples using the courier schedule published for each site in the synthetic study state.

10. Site Responsibilities [SEC-SITE]

Sites must complete local ethics acknowledgment and amendment training before activating a new protocol version for ongoing participants.

Inventory of PK kits must be confirmed against scheduled Day 1 visits. Extra kits on hand are tracked in the synthetic Twin.

Sites without validated overnight sample storage must not retain PK samples past the same-day courier departure.

11. Data-Collection Requirements [SEC-DATA]

Required EDC domains include demographics, dosing, PK collection times, ECG assessments, and laboratory shipment logs.

All EDC fields must use synthetic participant identifiers (P001–P005) and site identifiers (SITE-001–SITE-003).

A new EDC field is required under Protocol Version 2.0: `sample_processing_temperature_c` (Celsius), recorded for each PK and laboratory sample at the time of processing.

12. Appendix [SEC-APPENDIX]

Version history: Protocol Version 2.0 amends Version 1.0 with five controlled semantic changes for the AURORA-101 synthetic fixture.

Section identifiers (SEC-*) are stable anchors for evidence-linked extraction in Protocol 215.

End of Protocol Version 2.0.

Stable page map (generator contract): title=1, synopsis=2, objectives=3, design=4, eligibility=5, treatment=6, soa=7, day1_pk_fasting=8, ecg=9, lab=10, site=11, data=12, appendix=13.